Case study: George

Condition: **Stroke – left hemisphere ischaemic stroke**

72-year-old man · Acute stroke unit, public hospital

George Haddad is a 72-year-old man who was admitted to the acute stroke unit four days ago with a **left hemisphere ischaemic stroke**. He has dense weakness of his right arm and leg and difficulty with language: he understands simple sentences but can say only a few words. George lives with his wife, Mary, in a two-storey townhouse. Their daughter, Rania, visits every day. Before his stroke, George was independent, drove Mary to her appointments and spent most mornings in his vegetable garden with his grandchildren.

Speech pathology assessed George's swallowing on his second day of admission and recommended IDDSI Level 5 minced and moist food and IDDSI Level 2 mildly thick fluids, with George sitting fully upright and supervised for all food and drinks. A sign with these recommendations is taped to the wall above his bed, and speech pathology plans to review him again tomorrow. Nursing notes from this morning record that George "ate little breakfast" and that the family are unhappy with the hospital food.

You have been asked to review the positioning of George's right arm, after nursing staff noticed it is often found hanging over the side of the bed. When you arrive just after midday, George is semi-reclined in bed and slumped to the right. Rania is sitting beside him, feeding him pieces of a meat pie from the hospital café and holding a cup of water from his bedside jug to his lips. A large get-well card has been propped in front of the sign above his bed.

As you watch, George coughs after a mouthful of pie, then coughs again after a sip of water. When he tries to answer Rania, his voice sounds wet and gurgly. His right arm is hanging off the edge of the bed, and the back of his right hand looks puffier than his left. When you gently lift his arm back onto a pillow, he grimaces and pushes your hand away with his left hand.

Rania tells you her father "won't eat the hospital food" and is "starving". "Look at him, he's fine. He's just coughing because the pastry's dry," she says. She explains that he is sleepy "because of the tablets" and has been dozing on and off all morning. She then asks whether you know anything about residential aged care and "the aged care assessment", because she is worried her mother will not be able to look after him. She adds that Mary is adamant she wants George home and "won't hear of a nursing home".

There is no clear documentation of George's observations since this morning, including his temperature and oxygen saturation, or of whether the family have been given information about his swallowing recommendations. It is also unclear which medications George is taking, or whether his right shoulder has been assessed for pain.

George's nurse is attending to another patient two doors away. While you are talking, Rania breaks off another piece of pie and lifts it towards George's mouth. She hands you the television remote with her other hand and asks, "Can you fix this while you're here? He likes watching the cricket." At that moment, Mary arrives at the door carrying a container of home-made soup and a spoon, saying, "Here we go, George, some proper food."

You are the occupational therapy student reviewing George's arm positioning and your clinical supervisor is not currently in the room.

Present George's case to your clinical supervisor using the ISBAR framework. Identify the immediate concerns and explain what you would prioritise in responding to the situation.

ISBAR planning sheet

Use your 10 minutes of preparation to plan. Then speak for 7 minutes to your “supervisor”.

I Introduction

≈ 20 s

Who you are, your role, who the client is and why you need to talk now.

S Situation

≈ 40 s

A brief but complete summary of what is happening and the immediate concern.

B Background

≈ 1 min

Relevant history: the primary problem, medical and social history, medications/allergies, interventions so far and OT to date. Name missing information rather than assuming it.

A Assessment

≈ 2 min 20 s

Your interpretation. Case finding → what it may mean → consequence for safety and occupations → priority. Separate facts, interpretation, gaps and proposed assessment. What needs escalation?

R Recommendation

≈ 2 min 40 s

What should happen next, in priority order. Action → reason → who → review. Immediate safety, escalation, MDT input, one short-term and one longer-term OT assessment, intervention, referrals and follow-up.

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